

Response to HHS RFI

Accelerating the Adoption and Use of Artificial Intelligence as Part of Clinical Care

Docket: HHS Health Sector AI RFI

Submitted by: Mosaic Life Tech

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Submitter Information

Mosaic Life Tech is a healthcare AI governance advisory firm. Our principals have over 50 years of combined experience in health information technology, including leadership roles in federal health IT programs (ONC, CDC, ASTP), health information exchange operations serving over four million patients, and clinical informatics. We advise healthcare executives on establishing governance structures for AI-influenced clinical decisions.

We submit these comments based on direct participation in federal health IT policy development, industry governance forums, and review of recent survey data on AI adoption and oversight in healthcare settings.

Response to Question 3

Question: For non-medical devices, we understand that use of AI in clinical care may raise novel legal and implementation issues that challenge existing governance and accountability structures (e.g., relating to liability, indemnification, privacy, and security). What novel legal and implementation issues exist and what role, if any, should HHS play to help address them?

The most significant governance challenge is not legal ambiguity at the federal level. It is organizational ambiguity within health systems themselves.

Most health systems — and over 70% of acute care hospitals — now have AI tools influencing clinical decisions, whether through EHR-embedded algorithms, operational scheduling tools, or documentation assistants, though adoption drops sharply among independent, rural, and critical access hospitals. Many of these tools fall outside FDA device regulation. The question of "what federal rules apply" often has a clear answer. The question of "who inside our organization is accountable for this AI-influenced decision" frequently does not.

Recent survey data confirms a pervasive governance gap. The CHIME/Censinet AI Adoption Survey (December 2025) found that only 59% of healthcare organizations require formal approval before AI implementation, meaning over 40% lack even a basic

pre-implementation gate. Just 10% use automated monitoring to detect AI capabilities within their existing software, while 51% rely on informal, ad hoc discovery. ECRI has ranked "Insufficient Governance of AI in Healthcare" as the #1 health technology hazard for 2025, warning that failure to develop systemwide governance increases both patient safety and liability risk.

This pattern extends to federal healthcare settings. In January 2026, the VA Office of Inspector General reported that VHA had deployed generative AI tools without coordination with the National Center for Patient Safety, despite VHA Directive 1050.01 requiring such coordination to promote and safeguard patient safety. The OIG found no formal process to track AI-related safety issues. This illustrates how even large, sophisticated healthcare organizations can deploy AI without the governance structures needed to manage risk.

The governance gap is widening. *HTI-5's proposed removal of AI model card transparency requirements at § 170.315(b)(11) and significant reduction in DSI certification criteria effectively shifts the responsibility for evaluating AI safety, bias, and fitness-for-purpose from federal certification requirements to deploying healthcare organizations and the market.* This shift is navigable for well-resourced health systems with dedicated informatics leadership. For small and mid-market organizations, community hospitals, and safety-net providers, the shift creates a governance burden they lack capacity to meet.

The data confirms a "digital divide" in AI governance capacity. ASTP/ONC and AHA report that 86% of system-affiliated hospitals use predictive AI versus just 37% of independent facilities. HFMA survey data shows that among the 18% of health systems with mature AI governance, three-quarters have over \$1 billion in net patient revenue. Dr. Brian Anderson, CEO of the Coalition for Health AI, has noted that the digital divide is already being reinforced in the AI space, with rural communities lacking the IT staff to deploy and configure AI tools.

Vendor contracts address liability and indemnification from the vendor's perspective. But those contracts cannot resolve internal accountability when the organization hasn't decided who is responsible for validating AI tools against their patient population, monitoring for drift, or escalating concerns. A 2025 BMC Health Services Research study of AI implementation experts identified a "responsibility vacuum" where organizational structures obfuscate who should have accountability for AI monitoring. Only 22% of hospital leaders report high confidence they could produce a complete AI audit trail within 30 days if requested by a regulator.

HHS can help address this in several ways without adding regulatory burden:

First, HHS can support and amplify voluntary governance frameworks already emerging in the field. The Joint Commission and the Coalition for Health AI published the Guidance on the Responsible Use of AI in Healthcare on September 17, 2025. This guidance provides a reasonable structure for organizational accountability across seven elements: governance structures, patient privacy, data security, quality monitoring, voluntary safety event reporting, risk and bias assessment, and education and training.

Because the Joint Commission holds CMS deeming authority under 42 CFR Part 488, health systems are treating this guidance as a roadmap for aligning with likely future accreditation expectations. HHS recognition of such frameworks, without mandating them, would signal that establishing internal governance is expected and supported.

Second, HHS can consider technical assistance models that help under-resourced organizations build governance capacity. The Regional Extension Center program provides a clear precedent: it assisted approximately 134,000 primary care providers, and ONC evaluations found that 68% of REC participants achieved Stage 1 Meaningful Use versus just 12% of non-participants. A similar model for AI governance, potentially leveraging existing infrastructure at HRSA (through FQHCs), SAMHSA, or other agencies, could help smaller organizations develop oversight capabilities they currently lack.

Third, HHS can fund or disseminate research on effective governance structures. Health systems are experimenting with AI governance committees, chief AI officer roles, and cross-functional oversight bodies. KLAS Research has noted that "stronger organizational governance and ownership are needed to drive broader AI adoption" and that when governance provides a structured way to vet and integrate AI, adoption is more scalable and sustainable. Implementation science focused on AI governance, potentially through NIH or AHRQ, would help the field learn faster than each system discovering through trial and error.

The novel issue is not that AI creates new legal exposure. It is that AI creates decisions that cross traditional departmental boundaries, and most organizations have not adapted their governance structures to match. Federal clarity that organizational governance is expected, combined with support for building that capacity, would do more to accelerate responsible adoption than additional regulation.

Response to Question 5

Question: How can HHS best support private sector activities (e.g., accreditation, certification, industry-driven testing, and credentialing) to promote innovative and effective AI use in clinical care?

The private sector has already begun developing governance frameworks for AI in clinical care. HHS can accelerate adoption by supporting these efforts rather than duplicating them through regulation.

The Joint Commission, in collaboration with the Coalition for Health AI, published the Guidance on the Responsible Use of AI in Healthcare on September 17, 2025. This guidance addresses governance structure, validation, monitoring, training, and board visibility through seven core elements. Because the Joint Commission holds CMS deeming authority under 42 CFR Part 488 and accredits more than 80% of U.S. hospitals, health systems are paying attention to this guidance even though it does not create new accreditation standards. Legal analysts note that this guidance follows the Joint Commission's established pattern: the National Patient Safety Goals began as

recommendations before becoming accreditation requirements, and health equity guidance from 2023 became mandatory requirements effective January 2024.

Professional societies are also building AI-specific infrastructure. The American College of Radiology has launched Assess-AI, described as the world's first AI quality registry designed to monitor ongoing performance of imaging AI algorithms in real-world clinical settings. ACR's Recognized Center for Healthcare-AI (ARCH-AI) program provides a recognition framework for facilities meeting governance, model selection, acceptance testing, and monitoring criteria. ACR is working toward a formal AI accreditation program, with ACR Council approval anticipated for Spring 2027. The AMA's STEPS Forward toolkit provides practical governance guidance for health system leaders establishing AI oversight functions, including committee structure, decision rights, and escalation pathways. These efforts represent the field building governance from within, informed by clinical realities that federal regulation alone cannot capture.

HHS can support this work in several concrete ways:

HHS can publicly recognize voluntary frameworks as representing reasonable practice. This does not require endorsement or mandate. A statement that organizations following such guidance are taking appropriate steps toward responsible AI use would reduce uncertainty and encourage adoption.

HHS can support accreditation bodies in developing AI governance competencies within existing survey processes. Surveyors already assess governance, quality improvement, and patient safety. Integrating AI oversight into these existing frameworks, rather than creating separate AI-specific requirements, would reduce burden while improving accountability.

HHS can fund professional organizations and associations in developing training and credentialing for AI governance roles. Health systems are creating AI governance committees and oversight functions, but the people filling these roles often lack formal training in AI risk assessment or clinical validation methods. The AMA's STEPS Forward toolkit on AI governance provides a starting point, but broader workforce development would build capacity across the industry.

HHS can also support industry-driven testing and validation infrastructure. Health systems struggle to validate vendor claims about AI performance on their specific patient populations. Shared testing resources, benchmark datasets, or cooperative validation arrangements could help smaller and mid-sized systems that lack internal capacity for rigorous evaluation.

The risk of heavy federal regulation is that it freezes current practice into compliance requirements before the field has learned what actually works. The opportunity in supporting private sector frameworks is that governance can evolve with the technology while still establishing reasonable expectations for oversight.

Response to Question 7

Question: Which role(s), decision maker(s), or governing bodies within health care organizations have the most influence on the adoption of AI for clinical care? What are the primary administrative hurdles to the adoption of AI in clinical care?

No single role consistently owns AI adoption decisions across health systems. This fragmentation is itself a primary hurdle.

Industry leadership patterns reflect a structural divide between technical authority and clinical oversight. CHIME and AMDIS surveys document this dynamic: CIOs control technology infrastructure and serve on 63% of AI governance committees, but are typically not clinicians. CMIOs and CNIOs have clinical credibility but serve on only 45% of AI governance committees; AMDIS/WittKieffer surveys show that only 19% of CMIOs have a dedicated leadership budget, with 40% operating under the IT department's budget. Quality and patient safety leaders focus on outcomes but are rarely consulted during AI acquisition. Risk management addresses liability but often learns about AI deployments after the fact. Boards ask questions about AI oversight but receive inconsistent answers about who is accountable. The CHIME/Censinet survey found that 33% of organizations cite "unclear ownership" between IT, clinical, and compliance teams as a primary barrier to governance.

The CFO is often the decisive voice. HFMA's 2025 survey of 233 health system leaders found that CFOs and CIOs are the leading decision-makers when weighing AI vendor offerings, with cost reduction cited as the most frequent rationale for investment. Only 39% of CFOs believed AI investments would actually reduce overall costs, yet financial justification remains the primary gate. HHS's own RFI on accelerating AI in clinical care identifies reimbursement and payment policy as key levers, recognizing that AI adoption decisions frequently come down to financial justification. Yet CFOs typically lack the clinical or technical context to evaluate AI risk, creating a disconnect between the person who approves the investment and the people who understand what is being approved.

The organizations adopting AI most effectively have established clear governance ownership, typically through cross-functional AI governance committees with explicit decision rights, or through designated AI oversight roles that report to executive leadership. These structures do not slow adoption. They accelerate it, provided they include explicit decision rights and clear escalation pathways. Black Book Research found that health systems with AI governance councils are twice as likely to achieve ROI within 12 months, and programs with dashboards and clear ownership reach early ROI in 7.5 months versus 13.5 months without them. KLAS Research has observed that when governance provides a structured way to vet and integrate AI, adoption is more scalable and sustainable.

The primary administrative hurdle is not technology cost or complexity. It is the absence of a defined process for answering basic governance questions: Who approves the use of this AI tool? Who validates it against our patient population? Who monitors it after deployment? Who decides when to turn it off?

HHS could support adoption by funding research on effective governance structures and disseminating findings. Health systems are solving these problems independently, but there is limited sharing of what works across organizations. Implementation science focused on AI governance would help the field learn faster.

Closing

We appreciate the Department's focus on accelerating AI adoption while maintaining appropriate oversight. Across the governance challenges identified in Question 3, the private sector frameworks addressed in Question 5, and the organizational decision-making dynamics explored in Question 7, the evidence points consistently in one direction: governance is not a barrier to adoption but an enabler of it. Organizations with clear AI governance ownership achieve ROI faster and scale more effectively than those without. The most effective path forward supports organizational governance capacity rather than prescribing specific requirements. Health systems are ready to act. What they need from HHS is not more rules, but clear expectations and capacity support that help them govern AI responsibly.

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References

1. Office of the National Coordinator for Health Information Technology. Hospital Trends in the Use, Evaluation, and Governance of Predictive AI, 2023-2024. ASTP/ONC Data Brief. <https://healthit.gov/data/data-briefs/hospital-trends-use-evaluation-and-governance-predictive-ai-2023-2024/>
2. CHIME/Censinet. AI Adoption Survey Reveals Healthcare's Governance Gap and Drive Toward Agentic Usage. December 2025. <https://censinet.com/blog/ai-adoption-survey-reveals-healthcares-governance-gap-and-drive-toward-agentic-usage>
3. ECRI. Top 10 Health Technology Hazards for 2025. ECRI Institute. November 2024.
4. VA Office of Inspector General. Review of VHA's Use of Generative Artificial Intelligence. Preliminary Result Advisory Memorandum. January 2026. <https://www.vaog.gov/reports/preliminary-result-advisory-memorandum/review-vhas-use-generative-artificial-intelligence>
5. Assistant Secretary for Technology Policy / Office of the National Coordinator for Health Information Technology. Health Data, Technology, and Interoperability: ASTP/ONC Deregulatory Actions To Unleash Prosperity (HTI-5) Proposed Rule. 90 FR (Dec. 29, 2025). RIN 0955-AA09. 45 CFR § 170.315(b)(11). <https://www.federalregister.gov/documents/2025/12/29/2025-23896/health-data-technology-and-interoperability-astponc-deregulatory-actions-to-unleash-prosperity>
6. Anderson B. Quoted in: Healthcare AI Divide Threatens to Leave Safety-Net Providers Behind. Healthcare Dive. 2025. <https://www.healthcaredive.com/news/healthcare-ai-artificial-intelligence-digital-divide-safety-net-providers/801809/>
7. The Joint Commission and the Coalition for Health AI. Guidance on the Responsible Use of AI in Healthcare. September 17, 2025. <https://www.jointcommission.org>
8. Office of the National Coordinator for Health Information Technology. Evaluation of the Regional Extension Center Program: Final Report. 2016. https://healthit.gov/wp-content/uploads/2021/06/Evaluation_of_the_Regional_Extension_Center_Program_Final_Report_4_4_16.pdf
9. American College of Radiology. Assess-AI: AI Quality Registry. <https://www.acr.org>
10. American College of Radiology. ARCH-AI: Recognized Center for Healthcare-AI Program. <https://www.acr.org>
11. American Medical Association. STEPS Forward: AI Governance Toolkit. AMA Ed Hub. <https://edhub.ama-assn.org/steps-forward/module/2833560>
12. HFMA. AI Use Common Among Health Systems; Fleshed-Out Governance Not. Fierce Healthcare. 2025. <https://www.fiercehealthcare.com/ai-and-machine-learning/ai-use-common-among-health-systems-fleshed-out-governance-not>
13. Owens K, Griffen Z, Damaraju L. Responsibility Vacuum in AI Implementation. BMC Health Serv Res. 2025;25:1217. <https://link.springer.com/article/10.1186/s12913-025-13388-z>
14. Black Book Research. 2026 Health System & Hospital AI Governance Resource Guide. November 2025. <https://blackbookmarketresearch.com/governing-hospitals-ai-2026-board-to-bedside-accountability-guide>

15. KLAS Research. Healthcare AI Update 2025: What Use Cases Are Adopted the Most? Report #3912. 2025. <https://klasresearch.com/report/healthcare-ai-update-2025-what-use-cases-are-adopted-the-most/3912>
16. WittKieffer/AMDIS. Navigating Change & Growth: Insights into Physician Informatics Leadership. 2025. <https://wittkieffer.com/blog/the-evolving-healthcare-cmio-role>